

Farmakeio — Tirzepatide

GLP-1 Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.3 · generated 2026-08-09

Everything needed to prescribe Farmakeio tirzepatide, complete on its own. Values are copied literally into the Tebra Compound section — do not paraphrase, and do not adjust quantity, refill or days supply.

Farmakeio Pharmacy at a glance

Status	Active
Preferred states	AK, AL, AR, CO, CT, DC, DE, GA, HI, IA, ID, IL, IN, KS, KY, LA, MA, MD, ME, MI, MN, MO, MS, MT, NC, ND, NE, NH, NJ, NM, NY, OH, OK, OR, PA, RI, SC, SD, TN, UT, VA, VT, WA, WI, WV, WY
Ships to	AK, AL, AR, AZ, CO, CT, DC, DE, FL, GA, HI, IA, ID, IL, IN, KS, KY, LA, MA, MD, ME, MI, MN, MO, MS, MT, NC, ND, NE, NH, NJ, NM, NV, NY, OH, OK, OR, PA, RI, SC, SD, TN, TX, UT, VA, VT, WA, WI, WV, WY
CANNOT ship to	CA
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Farmakeio Pharmacy, 1736 N Greenville Ave, Richardson, TX 75081

Before you prescribe

- Products ship as a HOME KIT that includes syringes and supplies.
- Compounded with pyridoxine (B-6) for nausea prevention, not B-12.
- Semaglutide is a single concentration (2.5 mg/25 mg per mL). Dose is set by volume; the only variable on the Rx is how many mL.
- Tirzepatide is a single concentration (18 mg/25 mg per mL).

Farmakeio — Tirzepatide / B-6 Home Kit

Formulation: Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL — HOME KIT

Single concentration for every dose. Dose is set by volume.

Route: subcutaneous · **Frequency:** once weekly · **Order via:** Tebra Compound

Same for every dose

Drug Formulation	Tirzepatide/B-6 18mg/25mg per mL Home Kit
Select	Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Home Kit. Custom Rx for N/V mitigation, dosing flexibility, and added B-6.

4-week program

Dose	Name	Patient Instructions	Qty	RF	Days
2.5 mg	FARMAKEIO – Tirzepatide 2.5 mg – 4-Week Supply	INJECT 0.13ML (13 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	1	0	28
4.5 mg	FARMAKEIO – Tirzepatide 4.5 mg – 4-Week Supply	INJECT 0.25ML (25 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	1	0	28
7.5 mg	FARMAKEIO – Tirzepatide 7.5 mg – 4-Week Supply	INJECT 0.42ML (42 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	2	0	28
9 mg	FARMAKEIO – Tirzepatide 9 mg – 4-Week Supply	INJECT 0.50ML (50 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	2	0	28
13.5 mg	FARMAKEIO – Tirzepatide 13.5 mg – 4-Week Supply	INJECT 0.75ML (75 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	3	0	28
15 mg	FARMAKEIO – Tirzepatide 15 mg – 4-Week Supply	INJECT 0.83ML (83 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	4	0	28

8-week program

Dose	Name	Patient Instructions	Qty	RF	Days
2.5 mg	FARMAKEIO – Tirzepatide 2.5 mg – 8-Week Supply	INJECT 0.13ML (13 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	2	0	56
4.5 mg	FARMAKEIO – Tirzepatide 4.5 mg – 8-Week Supply	INJECT 0.25ML (25 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	2	0	56
7.5 mg	FARMAKEIO – Tirzepatide 7.5 mg – 8-Week Supply	INJECT 0.42ML (42 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	4	0	56
9 mg	FARMAKEIO – Tirzepatide 9 mg – 8-Week Supply	INJECT 0.50ML (50 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	4	0	56
13.5 mg	FARMAKEIO – Tirzepatide 13.5 mg – 8-Week Supply	INJECT 0.75ML (75 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	6	0	56
15 mg	FARMAKEIO – Tirzepatide 15 mg – 8-Week Supply	INJECT 0.83ML (83 UNITS) SUBCUTANEOUSLY ONCE WEEKLY	8	0	56

Vials dispensed

Dose	Units	Concentration	4-week	8-week
2.5 mg	13 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	1 ml	1 ml x 2
4.5 mg	25 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	1 ml	1 ml x 2
7.5 mg	42 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	2 ml	2 ml x 2
9 mg	50 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	2 ml	2 ml x 2
13.5 mg	75 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	3 ml	3 ml x 2
15 mg	83 u	Tirzepatide 18 mg / Pyridoxine (B-6) 25 mg per mL	4 ml	4 ml x 2

Pricing and charge codes

Provider and internal only. Never quote a price to a patient from this document without confirming with Operations. Includes: Telehealth visit, medication, shipping and supplies.

Tirzepatide — by dose tier

Tier	Doses	4-week (corp partner)	4-week (standard)	8-week
T1A	2 / 2.5 / 4 / 4.5 / 5 mg depending on pharmacy	\$349 FITTirzCP2	\$349 FITTirz002	\$599 FITTirzMT1
T2A	6.5 / 7.5 / 8.5 / 9 / 10 mg depending on pharmacy	\$399 FITTirzCP3	\$399 FITTirz003	\$649 FITTirzMT2
T3A	12.5 / 13.5 / 15 / 16 mg depending on pharmacy	\$449 FITTirzCP4	\$449 FITTirz004	\$799 FITTirzMT3

• Tirzepatide has no corporate discount. The corp partner code exists for attribution and tracking, not a lower price — it is the same figure as the standard 4-week at every tier. Do not present it to a corporate partner as a saving.

If the patient moves state

Patient state	Pharmacy
TX, NV, AZ, FL	Premier Pharmacy
CA	Belmar Pharmacy
Everywhere else	Farmakeio Pharmacy ← this document

• **Farmakeio Pharmacy cannot ship to:** CA. Those states must move to another pharmacy.
 • Established patients normally stay with their current pharmacy unless there is a clinical or supply reason to move, or the new state is one this pharmacy cannot serve.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**

Indications

• Weight loss
• Medically driven weight loss
• Increased energy
• Better sleep
• Improved mental health
• Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Program length and follow-up

INTERNAL ONLY. Internal and provider-facing only. Do not put this rule in a patient handout, the website, a pricing flyer, or any Circle post. A patient should not arrive expecting to graduate to 8-week.

Start rule	Every patient starts on the 4-week program. No exceptions.
Move to 8-week	A patient may be offered the 8-week program once they are stable at the dose they have landed on after escalation.
Who decides	Provider — provider-driven throughout. The 8-week program is an offer the provider extends, not a milestone the patient reaches or requests. Escalation, holding, reducing and weaning off are all provider discretion.
Visit cadence	4-week: Monthly provider visit. 8-week: Provider visit every 8 weeks. Either a video visit or an asynchronous visit. Both are acceptable for 4-week and 8-week follow-ups.
Weaning	Dose reduction and weaning off therapy are entirely at provider discretion. No fixed taper schedule is defined at the program level.

Reasons a patient stays at 4-week

- Cost — the 4-week program is the lower per-visit outlay and carries the discounted rate.
- Accountability — some patients want to see the provider monthly.
- Clinical — the provider judges that side effects are not adequately controlled and the patient needs to be seen more often.

Agent monograph — Tirzepatide — dual GIP and GLP-1 receptor agonist

Definition	A 39-amino-acid synthetic peptide based on the native GIP sequence, engineered to agonise both the glucose-dependent insulinotropic polypeptide (GIP) receptor and the GLP-1 receptor. A C20 fatty diacid moiety extends the half-life to roughly five days, allowing once-weekly dosing. FDA-approved as Mounjaro for type 2 diabetes and as Zepbound for chronic weight management and obstructive sleep apnoea in adults with obesity.
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Mechanism

- Dual incretin agonism — both the GIP and the GLP-1 receptor.
- GLP-1 component: glucose-dependent insulin secretion, glucagon suppression, delayed gastric emptying, central appetite reduction.
- GIP component: enhanced insulin secretion and improved insulin sensitivity in adipose tissue.
- GIP agonism may also blunt the nausea seen with GLP-1 activity alone, one proposed reason tolerability holds up at higher relative efficacy.

Clinical evidence

- SURMOUNT-1: mean weight loss around 20% of body weight at 15 mg over 72 weeks — the largest effect seen with a pharmacological agent in this class.
- SURPASS programme: glycaemic efficacy in type 2 diabetes, generally exceeding semaglutide in head-to-head comparison.
- SURMOUNT-OSA: supported the obstructive sleep apnoea indication.
- As with semaglutide, weight regain after discontinuation is well documented.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis — evaluate carefully before initiating
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to tirzepatide or any component of the formulation	• Active gallbladder disease or prior gallbladder-related surgical complications
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment (eGFR below 30 mL/min/1.73 m ²)
	• Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
	• History of suicidal ideation or active depression — monitor and ask

Medication interactions

• ORAL CONTRACEPTIVES: tirzepatide reduces the effectiveness of oral hormonal contraception. Advise a non-oral method, or add a barrier method, for four weeks after initiation and for four weeks after each dose increase. This is specific to tirzepatide and is easy to miss.
• Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
• Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
• Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
• ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. Anaesthesia guidance generally advises holding weekly GLP-1 therapy before an elective procedure. Tell the patient to disclose use to any surgeon, proceduralist or anaesthetist.
• Other GLP-1 or dual incretin agonists: do not combine.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Right-upper-quadrant pain — assess for gallbladder disease
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Contraceptive method if the patient uses oral hormonal contraception
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Mood and mental health
■ Gastrointestinal tolerance, especially through dose escalation	■ Lean mass preservation — protein intake and resistance training
■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis	■ Injection site and adherence

Patient counseling

“Tirzepatide works on two gut hormone pathways rather than one, which is why it tends to produce more weight loss than semaglutide. It slows stomach emptying and reduces appetite. Nausea is the most common side effect and is usually worst in the days after a dose increase. This is a compounded preparation rather than the brand-name product. If you take a birth control pill, it may not work as reliably for four weeks after we start and after each dose increase, so use a backup method. Weight tends to come back if the medication stops. Tell any surgeon or anaesthetist that you are taking this.”

- Gastrointestinal side effects are most common during dose escalation.
- Slower titration or holding at the current dose may improve tolerability.
- Instruct the patient to report persistent, severe or worsening symptoms.
- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on tirzepatide as a dual GIP/GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, reduced oral contraceptive effectiveness with the recommended backup precautions, the need to disclose use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	G47.33 — Obstructive sleep apnea (if documented and relevant)
E66.3 — Overweight	Z71.3 — Dietary counseling and surveillance
	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate. • Hypersensitivity or anaphylaxis-type reaction. • Pregnancy identified or confirmed. • Medullary thyroid carcinoma or MEN2 identified at any point. • Persistent severe vomiting with dehydration or acute kidney injury.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward. • Right-upper-quadrant pain, or gallstone symptoms. • Rapid or excessive weight loss, or loss of lean mass. • New or worsening depression, or any mention of suicidal thoughts. • Worsening retinopathy in a diabetic patient during rapid glycaemic improvement. • Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely. • Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.



Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Operations
- Clinical protocol questions — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute a pharmacy the tool marks as unavailable
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without confirming with Operations